Organised by
Supported by
22nd
Asian Breast Disease Association
(ABDA)
Annual Scientific Meeting
Strategising Multidisciplinary
Management of Breast Diseases
Poster ID: AA-040

CHASING THE SHADOWS OF BEAUTY : A CASE STUDY OF EXTENSIVE BREAST FILLERS MIGRATION



Amirah Azham[1], Su Fang Tan[1], Asiah Zukifly Apandi[1] ,Wan Mezlina[1]
[1]Radiology Department, Hospital Putrajaya, Putrajaya, Malaysia

INTRODUCTION

Breast augmentation has becoming a popular cosmetic procedure worldwide. Several techniques of breast augmentation are available in the market, including: implants, fat grafting, flap surgery, and injectable fillers. Among these options, injectable fillers have progressively increased their uses and notoriety due to the minimally invasive nature. However, migration of breast fillers beyond the intended injection site would be one of the possible complications, and this poses challenges in diagnosis and management. In this case study, we report a case of post-filler injection granuloma with extensive filler migration to both chest wall complicated with large right anterior abdominal wall siliconoma

CASE PRESENTATION

A 29-year-old lady presented with asymmetry right abdominal swelling two years post breast-filler injection. The body disfigurement also causes her mood instability and affect the family dynamic. Physical examination revealed palpable fullness over the right abdomen, no overlying skin changes.

IMAGING FINDINGS

She was first scheduled for ultrasound abdomen, which reported as a large cystic lesion within the subcutaneous tissues in the right lower abdominal wall. (Figure 1).Subsequently, a contrast-enhanced computed tomography (CECT) was done, which revealed: Bilateral breasts are dense and occupied by nodules of varying sizes in all quadrant. These nodules are isodense to the breast parenchyma and underlying muscles. A homogeneous non-enhancing lesion, along right lower anterior abdominal muscle extending to the right pelvic region, involving right rectus abdominis muscle. For diagnostic confirmation, she was scheduled for a magnetic resonance imaging (MRI) with silicon sequence. In this study, we noted innumerable small round and oval nodules of varying sizes seen scattered throughout the visualized breast parenchyma, periglandular fat and subcutaneous tissue. These nodules shows hypointense signal intensity on T1WI, hyperintense signal intensity on T2W/STIR. These nodules demonstrates silicone signal intensities on silicon only sequences and suppressed on silicon suppression images.(Figure 2 Green arrow) in keeping with silicone filler component. No obvious enhancement of these nodules on post contrast. Multiple nodules of similar appearance are seen in the aforementioned region of the right side anterior chest wall, tracking inferiorly from the bilateral chest wall into the bilateral abdominal wall and right inguinal region (Figure 3, Red arrow).

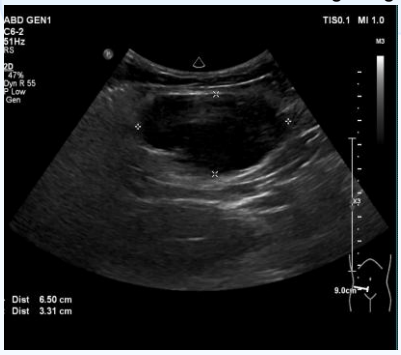


Figure 1. Ultrasound image of right lower abdomen



Figure 2. MRI - Silicone only

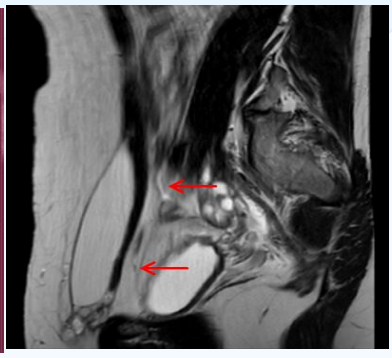


Figure 3. MRI T2WI Sagittal

DISCUSSION

Liquid injectable silicone has generated controversy over the decades. Despite absolute contraindication, free-silicone injection continues to be illegally performed for body contouring due to desirable aesthetic outcomes at a low cost.
Serious complications such as granuloma formation, migration, chronic cellulitis, infected fillers, abscess, pneumonitis, and death have also been reported. These severe complications can often be attributed to the use of unregulated or contaminated silicone injected in large volumes by non-medical personnel in non-clinical settings.

The ultrasound was non-diagnostic for silicone granuloma due to poor penetration and artefact from silicone. Contrasted CT scan is not the best modality as well due to the non specific features. MRI with silicon sequences remain the best modality for diagnostic purpose. Due to the extensive filler migration, this case is conservatively managed as it is very challenging for filler removal, as this may cause damage to the soft tissues and muscles and may require complex reconstruction.

CONCLUSION

Breast filler migration is potentially significant complication of cosmetic breast augmentation procedures. Knowledge of the radiologic characteristics and migration patterns of gel fillers and their related complications is useful for making an accurate diagnosis.

REFERENCES

1. Choi, Y.J., Lee, I.S., Song, Y.S. et al. Distant migration of gel filler: imaging findings following breast augmentation. Skeletal Radiol 51, 2223–2227 (2022). https://doi.org/10.1007/s00256-022-04037-1
2. Mingjie Huang et.al. Migration of breast augmentation filler to the mediastinum mimicking lymphoma: A case report. JTCVS Techniques 2022;14:155-8. https://doi.org/10.1016/j.xjtc.2022.06.002
3. Bhardwaj P, Greenwalt I, Ko K, Sher SR, Tsiapali E. Free Silicone Injections to the Breast: Delayed Complications and Surgical Management of Sequelae. Plast Reconstr Surg Glob Open. 2020 Nov 24;8(11):e3208. doi: 10.1097/GOX.0000000000003208. PMID: 33299689; PMCID: PMC7722574.

ACKNOWLEDGEMENTS

No conflict of interest.